STYLE AND TERMINOLOGY GUIDE

Clinical Document Standards — kelvaparib programme
Kelvaparib (KLV-3308)
Marnholt Oncology GmbH

1. PURPOSE AND SCOPE

This guide states the house conventions that apply to every clinical document prepared for the kelvaparib programme, including clinical study reports and their appendices, protocols and amendments, statistical analysis plans, synopses and periodic safety reports. It governs expression, terminology and presentation. It does not govern document architecture, which follows ICH E3 for clinical study reports and the applicable ICH guideline for other document types.

Where this guide and an ICH guideline both address a point, the guideline prevails and this guide is read as supplementing it. Where a departure from this guide is deliberate, the reason is recorded in the document review record.

2. DOCUMENT IDENTIFICATION AND VERSION CONTROL

- Every page carries the product name with its development code, the document type and the document date, in that order.
- Every page carries a page number in the form "Page n of N".
- The indication is stated on every page of documents that span more than one study.

3. COMPOUND, ARM AND TREATMENT NAMING

- The compound is named "kelvaparib" in body text. The development code KLV-3308 is used at first mention in a document, in page footers and on title pages, and is not repeated in body text thereafter.
- In a randomised study, arm labels state the agent and the dose, as in "Kelvaparib 300 mg". The comparator is named as it appears in the protocol, and is not abbreviated.
- In a dose-escalation study, cohorts are labelled by dose alone, as in "300 mg", because the agent is common to every cohort.
- Arm and cohort labels are identical in every table, figure, listing and narrative within a document set. A label is not shortened to fit a column.

4. TERMINOLOGY

- Adverse event terminology follows the coding dictionary in force. Verbatim terms are not used in narrative text where a coded preferred term exists.
- "Subject" is used throughout, not "patient", in clinical study reports and their appendices. "Patient" is permitted in documents written for a non-technical readership.

- "Treatment-emergent adverse event" is written in full at first mention in each section and may be abbreviated thereafter, provided the abbreviation appears in the list of abbreviations.
- British spelling is used, except within a coded term, a proper noun or a quotation, where the source spelling is preserved.

5. UNITS, NUMBERS AND PERCENTAGES

- Units follow the International System of Units. A space separates the value from the unit, as in "300 mg".
- Percentages are written as "per cent" in body text. The symbol is reserved for table column headers and for parenthetical values within a table cell.
- Percentages are reported to one decimal place. A percentage is suppressed where the denominator is fewer than five, and the denominator is stated wherever a percentage appears.
- Time is reported in the unit specified by the analysis plan, and the same unit is used for the same quantity throughout a document.

6. ABBREVIATIONS

- Every document contains a list of abbreviations, placed before the first numbered section. The list gives every abbreviation used in the document, including those in tables and figures.
- An abbreviation is expanded at first use in body text and thereafter used alone.
- An abbreviation is not introduced in a table or figure unless it also appears in the list of abbreviations.

7. CODING DICTIONARIES

- Every coded field states the dictionary and the version used to code it. The statement appears in the methods section of the document and as a footnote to every table presenting coded terms.
- Adverse events, medical history and indications are coded to the Medical Dictionary for Regulatory Activities. The version is stated to the point release, as in "MedDRA version 23.1".
- Concomitant and prior medications are coded to the WHO Drug Dictionary. The version is stated in the same form, as in "WHODrug Global B3".
- Where a document presents coded data from more than one data cut, the version

applying to each cut is stated separately. Versions in force for each study are recorded in the programme dictionary version log.

8. REPORTING OF CONCOMITANT MEDICATION

- Where concomitant medication data are collected, the clinical study report summarises them in the safety section, with the coding dictionary and version stated.
- The summary gives the number and proportion of subjects receiving each class, and identifies any medication prohibited by the protocol that was nevertheless received.

9. TABLES, FIGURES AND LISTINGS

- Numbering follows ICH E3: section 14.1 demographic and disposition, 14.2 efficacy, 14.3 safety, 14.4 subgroup analyses.
- A table title states the content, the population and, where relevant, the analysis set.
- Every table presenting coded terms carries a footnote naming the dictionary and version and stating the counting rule applied.
- A figure carries the same title conventions as a table and is numbered in the same series.

10. CROSS-REFERENCES

- A cross-reference names the section or output number and its title at first use.
- A cross-reference to a section that does not exist in the document is a defect and is corrected before issue rather than annotated.

11. REVIEW AND APPROVAL

Compliance with this guide is confirmed at document quality control before issue. A departure identified after issue is recorded, assessed for its effect on interpretation, and corrected at the next revision unless it affects a conclusion, in which case it is corrected by erratum.